



1. **12 lead criteria:** Any patient over 30 with S/S of pain between navel and jaw, chest pain/cardiac complaints, palpitations, S/S of ACS.
2. **Do electrical injuries, syncope events, and severe weakness qualify a rapid 12 lead?:** yes
3. **Does a patient with a previous or family history of cardiac issues require a 12 lead?:** Yes
4. **Do we place rapid 12 leads for patients with onset stroke symptoms and signs of an overdose?:** correct =M
5. **How many minutes after patient contact should we apply a rapid 12 lead?:** 5 mins
6. **What medication is indicated for suspected ACS?:** 324mg chewed aspirin
7. **What is our target SpO2 for suspected ACS?:** 92-96%
8. **When do we administer NTG in suspected ACS patients?:** SBP $\geq$ 180 mmHg and/or DBP $\geq$ 100 mmHg AFTER Fentanyl administration
9. **What is preload?:** volume of blood in the ventricles after diastole
10. **What is afterload?:** the pressure the left ventricle must overcome to eject blood during systole
11. **What is a total coronary occlusion?:** STEMI
12. **STEMI's account for:** 50-70% of heart attacks
13. **What is our scene time for STEMI alerts?:** less than 15 minutes